

QUANTIFYING HEALTH-SYSTEM SPECIALTY PHARMACISTS' INTERVENTIONS FOR PATIENTS UTILIZING EXTERNAL PHARMACIES

Laura Petry, PharmD, CSP; Kristen Whelchel, PharmD, CSP; Amy Mitchell, PharmD, CSP; Ryan Moore, MS; Autumn Zuckerman, PharmD, BCPS, CSP



CONCLUSIONS

- Health system specialty pharmacy (HSSP) pharmacists frequently assist non-HSSP patients to promote continuity of care and safe medication use
- An inability for the HSSP to dispense specialty medication results in significant uncompensated (> **\$3.25 million**) workload for HSSPs

BACKGROUND & PURPOSE

- Pharmacists at HSSPs are crucial in the complex coordination needed to ensure that patients receive their specialty medication safely and quickly.
- The purpose of this study was to quantify the time, intervention type, and labor costs related to HSSP pharmacists' documented actions performed as part of clinical care for non-HSSP patients.

METHODS

Setting	Academic medical center and health system specialty pharmacy (Vanderbilt Specialty Pharmacy)
Sample	Patients not filling a prescription with the HSSP who had at least one “quick action” documented by HSSP pharmacists using a flowsheet in the electronic health record
Outcomes	Descriptive analysis were performed to describe the number, type, and pharmacist time spent on actions taken on non-HSSP patients. Labor costs were calculated using an hourly rate including fringe of \$71.43

As part of normal workflow, pharmacists categorize actions taken, action outcome, and time spent on the action

Reviewed quick actions
over 3 months
July 1 to Sept 30, 2024

Calculated total estimated labor cost for time spent on actions for non-HSSP patients

EHR, electronic health record



RESULTS

Figure 1: Quick Action Categories Selected

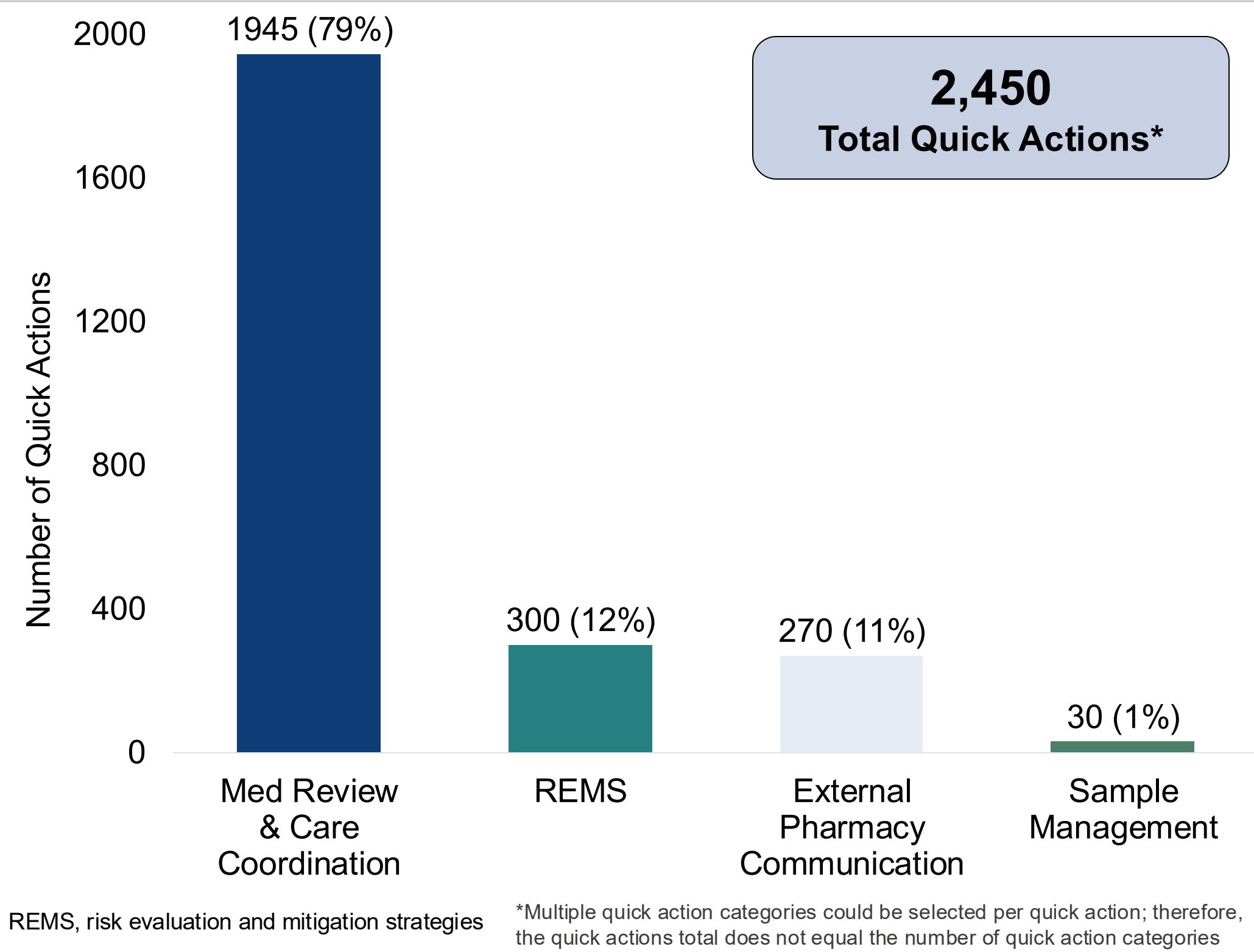


Table 1: Quick Actions Types Completed

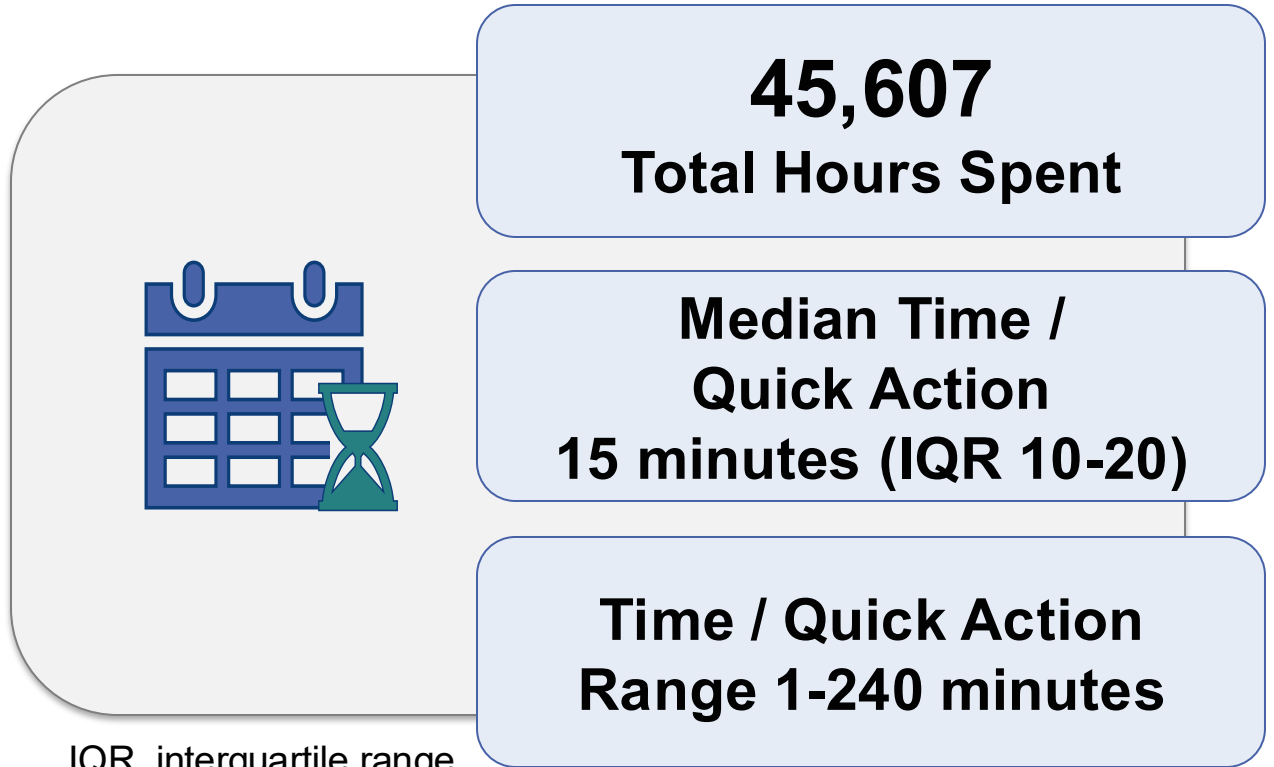
Category	Subcategory	Number
Medication Monitoring & Care Coordination	Med access coordination	1063
	Therapy monitoring- intervention	375
	Rx Prep	282
	Clinic communication - intervention	220
	Counseling provided	183
	Therapy monitoring - no intervention needed	101
	Ancillary med education	96
	Lab/Imaging coordination	63
	Clinic communication - no intervention	57
	Appointment coordination	34
External Pharmacy Communication	None selected	9
	Counseling declined	7
	Access issue	142
	RX confirmation	103
	Clinical info provided	61
	RX transfer	9
REMS Management	None selected	1
	REMS Refill	279
	REMS Enrollment	17
	External pharmacy communication	12
	REMS RX Triage	7
Sample Management	Sample Management	17
	Rep Meeting	14
	None selected	1
	Sample Inventory	1

Most Common Clinical Areas

Clinic	Number of Quick Actions
Oncology/Hematology	432
PAH/ILD*	308
Neurology	285
IBD	208
Pediatrics	202
Rheumatology	199

*PAH, pulmonary Arterial Hypertension; ILD, interstitial lung disease; IBD, inflammatory bowel disease

Time Spent



Costs

